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BAM TECHNICAL GUIDE | FACILITY COMPLIANCE SERIES

The Joint Commission's New Physical Environment Chapter: Where EC.02.05.01 EP 15 Went and What HVAC Surveyors Still Check in 2026

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> Effective January 1, 2026, the Environment of Care and Life Safety chapters are gone from the hospital manual. Fewer elements of performance does not mean fewer findings. Here is the HVAC crosswalk, the survey data behind it, and the documentation to have ready.

If your survey readiness binder still has tabs labeled EC.02.05.01 and LS.02.01.35, it is organized around a manual that no longer exists. Effective January 1, 2026, The Joint Commission replaced the Environment of Care (EC) and Life Safety (LS) chapters of the hospital and critical access hospital manuals with a single Physical Environment (PE) chapter, and then revised the two elements of performance that matter most for utilities (PE.04.01.01 EPs 1 and 2, along with PE.04.01.03 EP 3) again on March 1, 2026.

The headline number is that 48 percent of hospital EPs were eliminated. The number that matters for your ventilation systems is zero, because none of the underlying code requirements changed. This article walks through where the HVAC, pressure relationship, humidity, damper, and water management requirements moved, what surveyors scored in 2025, and what to have ready when the survey team schedules your OR suite walk for the morning of day one.

What Changed on January 1, 2026

The Joint Commission rebuilt its hospital requirements to line up with the structure of the CMS Conditions of Participation. The EC and LS chapters, which together carried 44 standards and roughly 460 elements of performance, were replaced by eight PE standards:

Standard	Subject
PE.01.01.01	Building is constructed, arranged, and maintained for patient safety
PE.02.01.01	Hazardous materials and waste

Standard	Subject
PE.03.01.01	Life Safety Code compliance (NFPA 101-2012)
PE.03.02.01	Protecting occupants when the Life Safety Code is not met or during construction (interim life safety measures)
PE.04.01.01	Building safety and facility management, including NFPA 99-2012 and equipment maintenance
PE.04.01.03	Utility systems, emergency power, and risk during repairs and maintenance
PE.04.01.05	Water management program for Legionella and other waterborne pathogens
PE.05.01.01	Imaging safety risks (CT, PET, nuclear medicine, MRI, fluoroscopy)

Requirements that went beyond CMS regulation moved to a new National Performance Goals (NPG) chapter, which also replaced the National Patient Safety Goals. Utility disruption procedures, for example, now live at NPG.11.03.01.

Two things did not change. The Ambulatory Health Care manual still uses the EC and LS chapters for now, so an ambulatory surgery center is still surveyed against EC.02.05.01 (The Joint Commission has signaled that the ambulatory manual may follow later). And the codes behind every deleted EP (NFPA 99-2012, NFPA 101-2012, NFPA 80 and 105-2010, and ASHRAE 170-2008 by reference) are still the codes.

The HVAC Crosswalk

Here is where the elements of performance facility directors quote most often ended up.

2025 requirement	2026 location	What it means in practice
EC.02.05.01 EP 15: ventilation in critical areas provides appropriate pressure relationships, air-exchange rates, filtration efficiencies, temperature, and humidity	Deleted as an EP. Scored under PE.04.01.01 EP 1 (NFPA 99-2012 compliance) or PE.03.01.01 EP 3, with the language retained in the Survey Process Guide	The requirement survives through NFPA 99-2012 Chapter 9, which references ASHRAE 170-2008
EC.02.05.01 EP 16: ventilation, temperature, and humidity in non-critical areas	Moved and revised to PE.04.01.01 EP 3: "proper ventilation, lighting, and temperature control in all pharmaceutical, patient care, and food preparation areas"	Pharmacies, nursing units, kitchens, labs
EC.02.05.01 EPs 3 through 8: utility inventory, high-risk components, ITM activities and frequencies, AEM criteria	PE.04.01.01 EP 2: "The hospital maintains essential equipment in safe operating condition," with documentation per manufacturer recommendations or an AEM program	Your air handler, exhaust fan, and damper PM schedules are still surveyable

2025 requirement	2026 location	What it means in practice
EC.02.05.05 EPs 2 and 4 through 8: testing and maintaining high-risk, infection control, and non-high-risk utility components	PE.04.01.01 EP 2	The 100 percent completion expectation for required ITM did not go away
EC.02.05.05 EP 1: managing air quality, infection control, dust, noise, and vibration risk during repairs and maintenance	PE.04.01.03 EP 4	Applies to coil cleaning, duct cleaning, and filter changes in occupied areas
EC.02.03.05 EP 18: fire and smoke dampers operated one year after installation and at least every six years	PE.04.01.01 EP 2; the Survey Process Guide document list places damper testing there	The six-year hospital cycle is still enforced through NFPA 80-2010 and NFPA 105-2010
EC.02.03.05 EP 28: documentation elements for fire protection testing	PE.04.01.01 EP 2 Note 1 since March 1, 2026 (EP 1 Note 3 from January 1), and PE.03.01.01 EP 3 Note 4: activity name, date, inventory of devices, required frequency, name and contact information of the person who performed it, NFPA standard referenced, results	The same seven data points, now attached to the maintenance EP
EC.02.05.02 EPs 1 through 4: water management program	PE.04.01.05 EPs 1 through 4	ASHRAE 188 and the CDC toolkit still referenced
LS.02.01.50 EP 2: HVAC complies with NFPA 101-2012 Section 9.2	PE.03.01.01 EP 3	Life Safety Code compliance as a whole
EC.02.06.01 EP 1: interior spaces safe and suitable	PE.01.01.01 EPs 1 and 2, with FGI Guidelines referenced for new, altered, or renovated space	The top-ranked physical environment finding in 2025
EC.04.01.01 EP 1: monitoring, reporting, investigating utility failures	NPG.11.01.01 EP 3	Your process for logging and investigating pressure and humidity failures

EP 15 Is Gone as an EP. The Requirement Is Not.

This is the point facility directors most need to hear from their compliance consultants and are not always hearing.

The 2026 Survey Process Guide's Physical Environment Document List and Review Tool keeps the EP 15 language almost word for word: in critical care areas designed to control airborne contaminants, the ventilation system provides appropriate pressure relationships, air-exchange rates, filtration efficiencies, temperature, and humidity, with the form and frequency of assessment per hospital policy. It adds that the basis for design compliance is the FGI Guidelines edition used at the time of design. The finding is written at PE.04.01.01 EP 1 or PE.03.01.01 EP 3.

The Joint Commission's current FAQ says the same thing from the other direction: NFPA 99-2012 Chapter 9 requires ventilation, temperature, and relative humidity to comply with ASHRAE 170-2008 for new, renovated, altered, or modernized areas, and existing spaces may remain as originally designed if the system has not been altered and is maintained to the original design parameters.

The consequence structure is also unchanged. The underlying Conditions of Participation are §482.41 (physical environment) and §482.42 (infection prevention). Under the old chapter, The Joint Commission's physical environment guidance described a ventilation finding in a critical area as a condition-level deficiency that triggered a Medicare deficiency follow-up survey within 45 days, reducible to a standard-level finding only if the organization repaired the process while surveyors were on site. The CoPs behind that have not changed.

What Surveyors Found in 2025

The Joint Commission stopped publishing percentage-based "most challenging standards" lists after the 2018 data. The last count-based data, from 1,330 hospital surveys in 2023, put EC.02.05.01 EP 15 fourth among all scored EPs in the higher-risk categories, with findings on 226 surveys, roughly one hospital in six.

For 2025, the January 2026 issue of Perspectives replaced counts with a ranked analysis weighted by frequency and severity. The top physical environment findings in hospitals were:

1. Maintaining interior spaces that are safe and suitable.
2. **Ensuring critical pressure relationships.**
3. Providing a clean, odor-free environment.
4. Properly handling and storing hazardous chemicals.
5. Maintaining furnishings and equipment.

Under pressure relationships, surveyors reported three recurring themes: incorrect pressure in operating rooms, sterile storage, and decontamination areas (negative where positive is required, or the reverse); temperature and humidity in ORs and sterile areas outside range (below 68°F or above 60 percent RH) with no corrective action or documentation explaining the deviation, found through both real-time checks and retrospective log review; and air exchange rates below minimum, with documentation missing or outdated and corrective actions not evident.

Read that list again. Every item is a maintenance outcome, not a design outcome. Those ORs were balanced correctly once.

The OR Suite Walk Happens on Day One

The 2026 building tour guidance instructs surveyors to review the OR suite early in the survey so the organization has time to correct while the team is still on site. Three items are on that walk: pressure relationships (checked live during the survey) and air exchange rates (from balance reports), scored at

PE.04.01.01 EP 1 for critical areas and EP 3 for non-critical areas; temperature and humidity levels; and surgical fire prevention.

There is one sentence in that guidance that changes how you should plan corrective work: the review of corrective action must include documentation that other areas supplied by the same air handler were not negatively impacted. Rebalancing one OR by stealing air from the next one is not a correction. Your fix has to be documented at the air handler level.

The Documentation to Have Ready

Document	What it should show	Who usually has it
Ventilation inventory for critical spaces	Every OR, AIIR, PE room, procedure room, SPD area, pharmacy, and the AHU serving each	Facilities
Design basis	ASHRAE 170 edition and FGI edition each space was designed to, with the design ACH, pressure, RH, and temperature	Facilities or the design engineer of record
Current test-and-balance report	Measured supply and exhaust cfm per space, computed ACH, date, and technician	TAB contractor
Pressure monitoring records	Method (BAS, permanent monitors, daily checks), frequency, out-of-range events, and corrective action	Facilities and infection prevention
Temperature and humidity records	Logs or trends covering nights and weekends, with excursion response	Facilities and surgical services
Filter records	Installed MERV per bank, change dates, pressure drop readings	Facilities or filter vendor
Utility ITM documentation	Activities, frequencies, completion, and results for air handlers, exhaust fans, dampers, and controls, per manufacturer or AEM	Facilities
Water management program	Team, diagram, risk plan, control limits, monitoring results, corrective actions, annual review	Water management team
Damper inspection reports	Location, date, inspector name and contact information, NFPA standard referenced, results, deficiency corrections	Facilities or damper contractor

If any row is blank, that is your survey finding, and you found it first.

Where Restoration Fits

Most of the 2025 pressure relationship and air change findings trace to the same mechanical causes: cooling coils fouled to the point of restricting airflow, final filters loaded past their pressure limit, exhaust ductwork in isolation areas that has never been cleaned, and building envelope leaks from cabling and renovation work. None of them require capital replacement. All of them require someone to measure, correct, and document.

That is the work we do. Our assessments produce measured before-and-after airflow, static pressure, and pressure differential data in a format written for the surveyor's document review, not for a marketing brochure. We covered the broader list of issues that trigger findings in our Joint Commission readiness guide; the 2026 chapter change moves the EP numbers, not the fixes.

Frequently Asked Questions

Did The Joint Commission eliminate the ventilation requirements for operating rooms and isolation rooms?

No. It eliminated the element of performance that restated them. The requirements come from NFPA 99-2012, which references ASHRAE 170-2008, and they are now scored under PE.04.01.01 EP 1 with the same language carried in the Survey Process Guide.

Which ASHRAE 170 edition does a surveyor hold my hospital to?

For spaces constructed or with plans approved on or after July 5, 2016, ASHRAE 170-2008 as referenced by NFPA 99-2012, or the state design requirement if it is more stringent. For older spaces, the design parameters of the edition in effect when the system was installed, provided the system has not been altered.

Are fire and smoke dampers still tested every six years?

Yes. The specific EP was deleted, but NFPA 80-2010 and NFPA 105-2010 remain in force through NFPA 101-2012 and NFPA 90A-2012, and PE.04.01.01 EP 2 requires the documentation. See our fire and smoke damper inspection guide.

Does my ambulatory surgery center follow the new PE chapter?

Not yet. The 2026 Ambulatory Health Care manual retains the EC and LS chapters, including EC.02.05.01 EP 7 for airborne contaminant control and EC.02.03.05 with a four-year damper cycle. Expect the ambulatory manual to be restructured later.

Do I still need to keep temperature and humidity logs?

The Joint Commission has no prescriptive daily logging requirement, but it expects critical rooms to be in constant compliance when in use and a reliable strategy to prove it. Non-continuous readings must be logged, and every non-compliant condition must be documented with corrective action. CMS interpretive guidance at A-0726 states that hospitals must maintain records demonstrating required temperature, humidity, and airflow levels.

What Happens Next

The chapter structure changed, the survey did not. If your last balance report predates your last coil cleaning, or your pressure monitoring stops at 5 p.m. on Friday, start here.

1. **Request a Joint Commission Readiness Review.** We inventory your critical spaces, pull the design basis, and measure current airflow, pressure, filtration, and coil condition against it.
2. **Receive a gap report organized by PE standard.** Findings are written in the 2026 chapter structure so they drop straight into your readiness binder.
3. **Close the gaps with documented restoration.** Coil restoration, filter correction, exhaust and duct cleaning, rebalancing, and post-work verification, including the air-handler-level documentation the building tour guidance now requires.

Contact us today to request your Joint Commission Readiness Review, or call (570) 900-5827.

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